

Patient Faq

Hypertension — patient FAQ

Derived from WHO *Be He@lthy Be Mobile — mHypertension handbook* (2020), the WHO *HEARTS technical package*, and *WHO treatment guidelines for hypertension*. Written for patients, not clinicians. Numbers and recommendations are simplified for everyday reading; see your health worker for advice that fits your specific situation.

The basics

What is high blood pressure?

Blood pressure is the force your blood pushes against the walls of your arteries as your heart pumps. **High blood pressure (hypertension) means that force stays above 140/90 most of the time.** It usually causes no symptoms — many people only find out by having their blood pressure measured at a clinic visit.

Why does it matter if I feel fine?

Because high blood pressure damages your arteries quietly for years before you notice anything. Over time it can cause heart attacks, strokes, heart failure and kidney failure. The good news: even small reductions in your blood pressure lower these risks.

Is hypertension curable?

No, but it is very manageable. With healthy habits and, if your health worker prescribes them, medications, most people can keep their blood pressure in a safe range for the rest of their lives.

What numbers should I aim for?

A safe target for most adults is **less than 140/90 mmHg**. Your health worker may set you a tighter target if you also have diabetes or kidney disease. If your reading is repeatedly higher than 140/90, see your health worker — do not wait for symptoms.

How often should I check my blood pressure?

- Have it checked at every clinic visit.
 - If you have been told your blood pressure is high, check more often. If you have a home machine that works correctly, your health worker can show you a schedule (often a few times a week).
 - If you ever get a reading much higher than usual *and* you feel unwell, do not wait — get help.
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Medication

Do I really need pills if I feel fine?

If your health worker prescribed them, yes. Pills work even when you do not feel anything happening. Stopping them because you “feel fine” is the most common reason people end up in hospital with a stroke or a heart attack.

Why am I taking more than one pill?

Most people with hypertension need two or more medicines from different families. Combining smaller doses works better and has fewer side effects than one large dose. Some “combination” tablets put two medicines into a single pill so you have less to keep track of — ask your health worker if that is an option for you.

What if I have side effects?

Side effects are common at the start and often go away after a few weeks. **Do not stop the medicine on your own.** Call or visit your health worker and describe the side effect — there is almost always an alternative they can switch you to.

What if I miss a dose?

Take it as soon as you remember the same day. If you have already started the next day, take only that day’s dose — do not double up. If you miss doses often, ask your health worker or pharmacist for a reminder app, a pillbox or a written schedule.

Can I take other medicines or herbs alongside my blood pressure pills?

Always ask first. Common painkillers (like ibuprofen), some cough and cold medicines and several traditional herbs can raise your blood pressure or interact with your medication. Bring everything you take to your next clinic visit.

Food and salt

How much salt is “too much”?

Less than **one level teaspoon (5 g) per day**, for everything you eat — not just what you sprinkle on top. Most adults eat more than double this without realising it.

Where is the hidden salt?

- Stock cubes, bouillon, soy sauce, fish sauce
- Bread, instant noodles, processed meat (sausages, ham)
- Salty fish, dried fish, salty snacks and pickles
- Canned soups, ready-made sauces, many flavoured crisps

Read labels where they exist. As a rule of thumb, the saltier it tastes, the more sodium it has.

What should I eat instead?

- **5 portions of fruits and vegetables per day** (one portion = one banana, mango or apple, or three tablespoons of cooked vegetables). Starchy roots like potato, sweet potato and cassava do not count toward the 5.
- Whole grains, beans and nuts. Fish (especially oily fish) at least twice a week.
- White meat (chicken, fish) more often than red meat. Limit red meat to once or twice a week.
- Cook with vegetable oils (sunflower, olive, soya, corn) rather than animal fat, palm oil or coconut oil.

- Boil, steam or bake instead of frying. Remove the chicken skin and the fatty parts of meat before cooking.

And sugar?

Cut back on sweet drinks, sugary tea and coffee, biscuits and cakes. These add empty calories that drive up weight, and weight gain pushes up blood pressure.

Movement

How much exercise do I need?

About **150 minutes of moderate activity per week** — that works out to 30 minutes a day, five days a week. “Moderate” means you breathe faster and your heart beats faster, but you can still hold a conversation.

What counts?

Brisk walking, cycling to work or to the shops, gardening, dancing, playing with children, sweeping or carrying water. You do not need a gym.

I have never exercised. Where do I start?

Start with 10 minutes at a time and build up. Two short walks today are already better than no walks today. Add one or two minutes per week.

Can I exercise if my blood pressure is very high?

If it is dangerously high (for example, above 180/110) and you have symptoms like chest pain or shortness of breath, **do not start an exercise programme** — see your health worker first. For most people with mild or controlled hypertension, regular moderate activity actually helps lower blood pressure.

Other lifestyle questions

Does smoking affect my blood pressure?

Smoking damages your arteries directly. Even if it does not change your blood pressure reading much, it multiplies the risk of heart attack and stroke that comes from your hypertension. Quitting is the single most powerful thing you can do for your heart. If you smoke, ask your health worker about a tobacco-cessation programme.

What about alcohol?

The safest amount is none. If you do drink, aim for **fewer than 2 drinks on any day and at least 2 alcohol-free days per week**. Never drink: when driving, when operating machinery, when pregnant or breastfeeding, or when you are taking medicines that interact with alcohol.

Do I need to lose weight?

If you are overweight, even losing a few kilograms can lower your blood pressure noticeably. Aim for a slow, steady loss (about 0.5 kg per week) through eating habits and movement together — crash diets do not last.

Can stress cause hypertension?

Long-term stress contributes, but it is not the main cause. Managing stress (sleep, breathing exercises, talking to people you trust) is helpful, but it does not replace medication if you have already been diagnosed.

Warning signs

When should I go to the hospital straight away?

Any of these together with a high reading is a **hypertensive emergency**:

- Severe headache that does not go away
- Loss of consciousness, confusion or sudden weakness
- Chest pain or pressure
- Nausea and vomiting
- Severe dizziness
- Sudden blurred vision or vision loss
- A very fast or pounding heartbeat
- Sudden trouble speaking or one side of the face drooping

Do not drive yourself. Call your local emergency number or have someone take you to the nearest hospital.

What about a high reading on its own, without symptoms?

If a single reading is very high (for example over 180/110) but you feel well, sit quietly for 5 minutes and check again. If it is still very high, contact your health worker the same day. If you also feel unwell, go to hospital.

Living with hypertension

How long will I need to take the medication?

For most people, indefinitely. Hypertension is a long-term condition, not an infection that clears up. The reward for sticking with it is many extra years of healthy life.

Can I ever come off the pills?

Sometimes, if you reach a healthy weight, eat well, exercise regularly and quit smoking, your health worker may try lowering your dose. **Never reduce or stop on your own** — it must be a clinical decision, with follow-up readings.

My parent had a stroke. Am I at higher risk?

Yes, family history is a risk factor. That makes it even more important to know your numbers, check your blood pressure regularly and act on the lifestyle steps above.

Can my family help?

Yes — and they should. Cooking with less salt as a household, eating more vegetables together, walking together, and supporting you to take your medication on time all make a real difference. Hypertension management is easier when it is a family habit rather than a personal restriction.